POST-OPERATIVE DISCHARGE INSTRUCTIONS

Patient: SMITH, ROBERT T. MRN: 77120458 DOB: 09/02/1961
Procedure: Left total knee arthroplasty (TKA) Date of Surgery: 10/27/2026
Surgeon: K. Okafor, MD Anesthesia: Spinal w/ adductor canal block

POST-OP DIAGNOSIS: Severe primary osteoarthritis, left knee.

WOUND CARE: Maintain Aquacel dressing x 7 days unless saturated. Do not submerge incision (no tub, pool, or hot tub) until cleared at post-op visit. Monitor for erythema, purulent drainage, dehiscence, or warmth at the incision site.

WEIGHT BEARING: WBAT (weight bearing as tolerated) LLE w/ front-wheeled walker. Perform home exercise program TID. Do not place a pillow under the knee; keep the knee extended when resting.

DVT PROPHYLAXIS: Aspirin 81 mg PO BID x 35 days. Wear TED stockings on both legs during the day. Perform ankle pumps hourly while awake.

PAIN MANAGEMENT:
- Acetaminophen 1000 mg PO q8h scheduled
- Oxycodone 5 mg PO q6h PRN severe breakthrough pain (max 4 doses/24h)
- Celecoxib 200 mg PO QD x 14 days
- Docusate 100 mg PO BID while taking opioid (prevent constipation)

Do not drive or consume alcohol while taking oxycodone.

CALL THE OFFICE (555-0142) IF: calf pain/swelling, fever >101.5F, increasing redness or drainage, pain uncontrolled by medication.
CALL 911 IF: chest pain, sudden shortness of breath, or coughing up blood (possible pulmonary embolism).

FOLLOW-UP: Staples removal and wound check at 14 days. Outpatient PT 3x/week beginning 10/30/2026.